

SPECIAL ARTICLE

People-Centered Language Recommendations for Sleep Research Communication

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The growing embrace of patient-centered outcomes research (PCOR) in sleep medicine is a significant step forward for the field. In engaging and incorporating the unique perspectives of people with sleep disorders, PCOR enhances the relevance of findings and facilitates the uptake of research into practice. While centering research design around what matters most to people with sleep disorders is critical, research communication must be similarly people-centered. One approach is using “people-centered language” in both professional and public communications. People-centered language is rooted in sociolinguistic research demonstrating that language both reflects and shapes attitudes. People-centered language puts people first, is precise and neutral, and respects autonomy. By adhering to the language guidelines described in this article, sleep researchers will better serve the field’s most important stakeholders.

Keywords: Narcolepsy, OSA, insomnia, circadian rhythm disorders, restless legs syndrome, REM sleep behavior disorder, communication, language, terminology, patient-centered outcomes, stigma.

Statement of Significance

Guidance for improving the language researchers use to talk to and about people with studied health conditions has been issued in several fields, including psychology and endocrinology. This article is the first to develop such recommendations for the sleep research field. Additionally, these guidelines combine people-first terminology with language considerations around neutrality and autonomy into a novel and comprehensive “people-centered” approach.

INTRODUCTION

The sleep medicine community is increasingly recognizing the importance of patient-centered outcomes and partnerships in research.^{1,2} In step with these initiatives, sleep researchers should use thoughtful and purposeful language in both professional and public communication. Such language should be *people-centered*, respecting the humanity and dignity of people who participate in research and people with sleep disorders generally.

Beyond a matter of political correctness, people-centered language is grounded in sociolinguistic principles positing that language significantly shapes ideological preconceptions.³ In other words, language not only reflects attitudes, it also affects them. Language choices in medical research communication can thus unintentionally perpetuate stereotypes and negative attitudes about people with the studied conditions. Since many people with sleep disorders face stigma,⁴⁻⁶ language is indeed a consequential consideration for sleep researchers. Language also shapes perceptions of research and can either discourage or empower people to participate.⁷

ELEMENTS AND EXAMPLES OF PEOPLE-CENTERED LANGUAGE

Put People First

People-first terminology literally puts the word “people” or “person” before the name of their condition. People-first terminology also avoids equating people with their condition by linking the two with words like “have” or “with” rather than “is” or “are.” Using people-first terms (eg, “people with narcolepsy”) instead of condition-first (eg, “narcolepsy patients”) or condition-only (eg, “narcoleptics”) terms signals that a diagnosis is something a person has, not a person’s defining characteristic.

In a study of how people-first terminology affects attitudes, participants displayed less tolerant views of people referred to as “the mentally ill” than those referred to as “people with mental illness.”⁸ For example, participants were more likely to agree with

the statement “the mentally ill should be isolated from the community” than the statement “people with mental illnesses should be isolated from the community.” This difference occurred even though the definition of “the mentally ill” or “people with mental illness” at the top of each survey was identical. Similarly, compared to “person with epilepsy,” “epileptic” provoked more negative attitudes toward people with epilepsy in two studies.^{9,10}

There are also studies demonstrating that people with health conditions prefer people-first terminology. People with multiple sclerosis preferred the term “people with MS” be used in scientific journals.¹¹ Adults seeking weight-related treatment reported feeling less stigmatized when physicians refer to their “weight” or “weight problem” rather than labeling them as “obese” or “overweight.”¹²

Be Precise and Neutral

Clichés like “suffering from,” “afflicted with,” and “victim of” have strong emotional overtones. For example, describing people as “suffering from” their condition or symptoms implies a deep and perpetual state of torment.¹³ Suffering is a self-descriptive concept that one should not assume applies to another person or a group of people.^{14,15} Describing what people “have” or “experience” (eg, “people with idiopathic hypersomnia experience excessive daytime sleepiness” rather than “people with idiopathic hypersomnia suffer from excessive daytime sleepiness”) is more precise and avoids imposing superfluous meaning.

“Burden” is a value-laden word when discussing the effects of a sleep disorder on healthcare costs or on families.¹⁶ “Impact” or “costs” are more neutral terms to use in these contexts (eg, “the economic impact of sleep apnea” rather than “the economic burden of sleep apnea”).

Respect Autonomy

Certain terms reinforce uneven power dynamics in medicine and research rather than affirm the autonomy of people in

their own care. For example, there is growing agreement that referring to treatment “adherence” rather than “compliance” better reflects a culture of shared decision-making.^{17,18} “Compliance” is also conceptually flawed in its implication that following a treatment regimen is a matter of obedience or recalcitrance, ignoring the social, economic, and other factors involved.¹⁹

Labeling self-reported symptoms as “subjective” connotes unreliability, overemphasizing the relative objectivity of physician observations.²⁰ For example, “self-reported sleep quality” is preferable to “subjective sleep quality.”

Using the term “participant” rather than “subject” expresses respect for the people who agree to take part in studies and recognizes their autonomy in doing so.²¹

CONCLUSION

In determining whether improving language in sleep research is a practical goal, one should consider the moves toward people-centered language in other research areas. In 1992, the American Psychological Association issued language guidelines for its journals that include using people-first terms (eg, “people with schizophrenia” rather than “schizophrenics”) and emotionally neutral language (eg, “individual who had a stroke” rather than “stroke victim”).²² Likewise, the American Medical Association Manual of Style asks authors to avoid equating people with their conditions (eg, “epileptics”) and using emotional phrases (eg, “suffering from”).²³ Since 1998, *BMJ* editorial policy has replaced the term “subjects” with “participants” wherever appropriate.²⁴

Sleep researchers may worry that adopting people-centered language will be onerous or hinder the use of elegant shorthand. However, convenience should not take priority over reducing stigma and better engaging the people this research is intended to serve.

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ACKNOWLEDGMENTS

This article was partially funded through a Patient-Centered Outcomes Research Institute (PCORI) Award (EAIN-3394-UOA). The opinions presented in this article are solely the responsibility of the author and do not necessarily represent the views of the Patient-Centered Outcomes Research Institute (PCORI), its Board of Governors or Methodology Committee.

SUBMISSION & CORRESPONDENCE INFORMATION

Submitted for publication February, 2017

Submitted in final revised form March, 2017

Accepted for publication March, 2017

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